HEALTH SYSTEM

Community Medicine (PSM) | 3rd Year

Hi Sammy!
India's health system =
SUPER high yield in PSM!
Tables are everything here!

Health System | PSM | 3rd Year | MBBS

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Healthcare Delivery Levels + Infrastructure

3-tier system = asked
in EVERY PSM exam!
Know the levels cold!

HEALTHCARE DELIVERY

Organized system of providing health services to the population at primary, secondary & tertiary levels.

Three-Tier Healthcare System

Level	Facilities	Key Features
Primary	SC, PHC, HWC	First contact, preventive + curative, community-based
Secondary	CHC, SDH, FRU	Specialist care, referral from primary, 30-bed CHC
Tertiary	DH, Medical College	Super-specialist care, teaching, research, 75-500 beds

Indian Public Health Standards (IPHS)

- IPHS = minimum standards for govt health facilities (revised 2012, updated 2022)
- Two levels: Essential (must have) and Desirable (aim for)
- Goal: 50% IPHS compliance by 2025-26, score >80% = compliant
- New norm: SC within 30 min walk from habitation in hilly/desert areas

Health + Wellness Centres (HWC)

- Under Ayushman Bharat (2018) — upgradation of SC/PHC to HWC
- Target: 1,50,000 HWCs across India (rural + urban)
- Services: Comprehensive Primary Healthcare (CPHC) — 12 service packages

★ Remember: SC -> PHC -> CHC -> DH
Population: 5K -> 30K -> 1.2L -> District | Beds: 0 -> 6 -> 30 -> 75-500
Each PHC covers 6 SCs, each CHC covers 4 PHCs. Simple ratio!

PHC, CHC, SC, DH — Staff + Norms

This TABLE is asked in every single PSM exam! Memorize it perfectly!

Health Facility Comparison (THE Table)

Facility	Pop (Plains)	Pop (Hilly)	Beds	Key Staff
Sub-Centre	5,000	3,000	None	ANM + Male Health Worker
PHC	30,000	20,000	6	MO (MBBS), Staff Nurse, Pharmacist
CHC	1,20,000	80,000	30	4 Specialists + 6 GDMOs + 19 Nurses
District Hosp	Per district	Per district	75-500	Full specialist services

Sub-Centre (SC) — Details

- Most peripheral & first contact point in rural areas
- Staff: 1 ANM (essential) + 1 Male Health Worker (MPW)
- Services: Maternal care, immunization, family planning, basic drugs
- Linked with: ASHA, AWW (Anganwadi Worker), Village Health Guide

Primary Health Centre (PHC) — Details

- First port of call to a qualified doctor in rural areas
- Referral unit for 6 Sub-Centres | 6 beds (in-patient)
- Staff: MO (MBBS) x1-2, Staff Nurse, Pharmacist, Lab Tech, Health Worker
- 24x7 PHC: 2 MOs + 7 Staff Nurses + 2 Lab Techs (IPHS 2022)

Community Health Centre (CHC) — Details

- 30-bedded hospital — covers 4 PHCs
- 4 Specialists: Surgeon, Physician, OBG, Paediatrician
- Also: Anaesthetist, Dental Surgeon, 6 GDMOs, 19 Staff Nurses
- Functions as First Referral Unit (FRU) for obstetric emergencies

★ **District Hospital = highest referral centre at district level (75-500 beds)**
At least 1 per district. Full specialist + super-specialist services.

NHM = NRHM + NUHM!
So many schemes but
years & details matter!

National Health Mission (NHM) — Overview

NHM:	Launched 2013 — umbrella for NRHM + NUHM
NRHM:	Launched 12 April 2005 — rural health focus
NUHM:	Launched 2013 — urban poor health focus
Goal:	Universal access to equitable, affordable, quality healthcare
Focus:	EAG states + NE states given special priority

NRHM vs NUHM — Comparison

Feature	NRHM (Rural)	NUHM (Urban)
Year	2005	2013
Target	Rural population	Urban poor / slum dwellers
Facilities	SC, PHC, CHC	U-PHC, U-CHC, UHWC
Key worker	ASHA (1 per 1000)	Urban ASHA (1 per 1000-2500)
Strategy	3-tier system strengthening	Partnership with municipal bodies

Ayushman Bharat (2018)

1. Two components: (1) Health & Wellness Centres + (2) PM-JAY
2. HWC: 1,50,000 centres for Comprehensive Primary Healthcare (CPHC)
3. PM-JAY: Rs 5 lakh/family/year for secondary & tertiary hospitalization
4. World's largest publicly funded health assurance scheme
5. 36+ crore Ayushman Cards created (as of 2024)

Important Schemes Under NHM

Scheme	Key Details
Janani Suraksha Yojana	2005 — cash incentive for institutional delivery, reduce MMR
Janani Shishu Suraksha	Free delivery + C-section + sick newborn care in govt hospitals
RBSK	Rashtriya Bal Swasthya Karyakram — child health screening (0-18 yrs)
RKSK	Rashtriya Kishor Swasthya Karyakram — adolescent health (10-19 yrs)
Free Diagnostics	Free diagnostic services at public health facilities
PM Dialysis Program	Free dialysis at district hospitals

ASHA & ANM roles =
always asked in exams!
Know the differences!

ASHA — Accredited Social Health Activist

Launched:	Under NRHM (2005) — community health volunteer
Selection:	Married woman, 25-45 yrs, from same village, 10th pass
Coverage:	1 ASHA per 1000 population (rural)
Payment:	Performance-based incentive (not fixed salary)
Role:	Link between community & health system
Functions:	Health awareness, ANC registration, institutional delivery Immunization mobilization, DOTs provider, ASHA drug kit

ANM — Auxiliary Nurse Midwife

- Posted at Sub-Centre — key functionary of primary healthcare
- Trained health professional (2-year ANM course)
- Conducts: ANC checkups, immunization, family planning, deliveries
- Village Health & Nutrition Day (VHND) — conducted by ANM monthly
- Works with: ASHA, AWW (Anganwadi Worker), MPW

ASHA vs ANM — Quick Comparison

Feature	ASHA	ANM
Type	Community volunteer	Trained health professional
Qualification	10th pass minimum	ANM course (2 years)
Payment	Performance-based incentive	Fixed government salary
Posted at	Village level	Sub-Centre
Coverage	1 per 1000 pop	1 per Sub-Centre (5000 pop)

India's Health Expenditure — Key Facts

Indicator	Data
Total Health Expenditure (THE)	~3.3% of GDP (NHA 2019-20)
Government Health Expenditure	~1.35% of GDP (target: 2.5%)
Out-of-Pocket Expenditure (OOPE)	~47.1% of THE (declining trend)
NHM Funding pattern	60:40 (Centre:State); 90:10 for NE states
WHO recommendation	Min 5% of GDP on health

Health System = easy
20+ marks if you know
the tables! Go Sammy!

Most Repeated LAQs (10–12 marks)

1. Describe the healthcare delivery system in India with levels & facilities
2. Write about PHC — population norms, staffing pattern, functions (IPHS)
3. National Health Mission — components, objectives, achievements
4. Ayushman Bharat — two components, PM-JAY coverage & eligibility
5. Role of ASHA in National Rural Health Mission

Most Repeated SAQs (5–6 marks)

1. Compare PHC vs CHC (table format — staff, beds, population)
2. ASHA — selection criteria, roles, incentives
3. Sub-Centre — staff, services, population norms
4. NRHM vs NUHM — differences in approach
5. Health & Wellness Centres under Ayushman Bharat
6. Out-of-pocket expenditure — why it matters

One-Liner Associations (MCQ/Viva Gold)

Question	Answer
Most peripheral health facility	Sub-Centre
First contact with qualified doctor (rural)	PHC
Population norm for PHC (plains)	30,000
Number of beds at CHC	30
4 Specialists at CHC	Surgery, Medicine, OBG, Paediatrics
NRHM launched year	2005 (12th April)
NHM = ?	NRHM (2005) + NUHM (2013)
Ayushman Bharat PM-JAY cover	Rs 5 lakh/family/year
ASHA coverage	1 per 1000 population
ASHA payment type	Performance-based incentive
India's OOPE on health	~47% of total health expenditure
WHO-recommended health spend	Minimum 5% of GDP

♥ You're going to work IN this health system one day, Sammy!
Understanding it now = being a better doctor later. So proud of you!